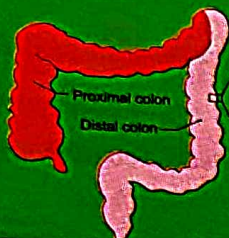


CHAPTER

5



Atisara and Pravahika

Learning Objectives

Atisara

- ⇒ To learn the concept of Atisara
- ⇒ To know about the Pratyatma Lakshana of Atisara and Pravahika
- ⇒ To learn the Nidana Panchaka of Atisara described in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka based on the progression of disease
- ⇒ To describe Sadhyasadyata of Atisara with current understanding from western physiology point of view
- ⇒ To describe Nivritta Atisara/ Vigata Atisara Lakshana along with western physiological understanding
- ⇒ To describe the Samprapti of Pravahika
- ⇒ To differentiate between Atisara and Pravahika
- ⇒ To differentiate Doshaja Atisara

Pravahika

- ⇒ To learn the concept of Pravahika
- ⇒ To learn about the Pratyatma Lakshana (cardinal feature) of Pravahika
- ⇒ To learn the Nidana Panchaka of Pravahika mentioned in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka based on the progression of the disease
- ⇒ To learn about Samanya Lakshana and Vishishta Lakshana of Pravahika
- ⇒ To know the difference between Atisara and Pravahika
- ⇒ To learn about Bhayaja, Shokaja and Raktaja Atisara and its current understanding

Atisara

Introduction

Excretion is one of the normal physiological function sequentially seen after the process of digestion. It signifies the elimination of waste products which are formed in the large intestine after the absorption of water, electrolytes and other nutrients.

Atisara is a spectrum of diseases where in the individual will pass liquid to semi-solid stools with increased frequency.

This occurs when the person is having hypermotility of large intestine, where the reabsorption of water and

electrolytes is hampered.

In Atisara, usually the person will have Agnimandya (loss of appetite) which leads to the excess formation of Mala rather than Sara portion from ingested food. The pathological event is responsible for the manifestation of Atisara.

This is considered as Ashukari Vikara (producing acute manifestation) involving Purishavaha Srotas. The type of Srotodushti involved is Atipravritti.

As far as Dhatu affliction is concerned, Rasa Kshaya is observed which can be evident in the form of sunken eyes, loss of skin turgor and altered state of consciousness in case of severe form of the disease manifestation.

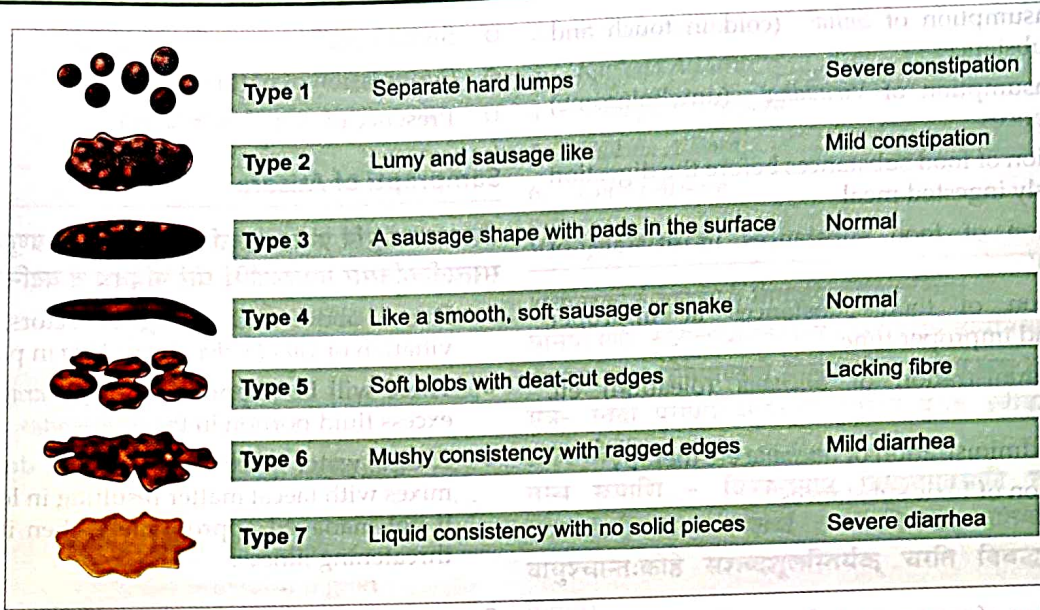


Figure 8: Bristol stool chart for consistency of faecal matter

Pratyatma Lakshana of Atisara

गुदेन बहुद्रव- सरणमतिसार इत्यर्थः। Ma.Ni. – 3/4, Madhu
Excess passage of loose or liquid or semi-solid stools through anal orifice is termed as *Atisara*. This is occurring due to the increased peristaltic movement or poor large intestinal obstruction.

Atisara Nidana

गुर्वतिस्निग्धरुक्षोष्णद्रवस्थूलातिशीतलैः।
विरुद्धाध्यशनाजीर्णैर्विषमैश्चापि भोजनैः॥1॥
स्नेहाद्यैरतियुक्तैश्च मिथ्यायुक्तैर्विषैर्भयैः।
शोकाहुष्टाम्बुमद्यातिपानैः सात्यतुर्पर्ययैः॥2॥

जलाभिरमणैर्वेगविघातैः क्रिमिदोषतः।

नृणां भवत्यतीसारो लक्षणं तस्य वक्ष्यते॥3॥

Ma.Ni. – 3/1-3

- Excess consumption of *Guru*¹ (substances which are heavy to digest) substances
- Excess consumption of *Atisnigdha*² (excessively unctuous) substances
- Excess consumption of *Ruksha*³ (dry) substances
- Excess consumption of *Ushna*⁴ (hot in potency) substances
- Excess consumption of *Drava*⁵ (liquid) substances
- Excess consumption of *Sthula*⁶ (large in size) substances

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- 1 When an individual consumes *Guru* substances in more quantity, it takes more time to get complete digestion. When there is more time taken for the digestion to occur, it may result in infection, as most of the Indians are having the carbohydrate rich diet and bacteria proliferate rapidly in such carbohydrate rich diet. This leads to the inflammation of the alimentary canal, thus the person may have rapid peristaltic movements leading to the manifestation of diarrhoea.
- 2 When an individual consumes more amount of unctuous substances, that will lead to covering of the intestinal mucosa with unctuous substance and there will be increased peristaltic movements because of its slippery nature. This will lead to the excess movement of liquid or semiliquid stools from the anus. The same thing can occur when an individual is under one of the *Purvakarma* procedure i.e. *Snehapana* therapy.
- 3 When an individual consumes more amount of *Ruksha* (dry) substances frequently, this may stimulate excess mucus secretion from surface mucous cells of the stomach. This mucus is very viscous which coats the stomach wall of 1 mm thick and it is alkaline in nature. This will neutralise the hydrochloric acid, and this may further lead to the similar mechanism of formation of *Ama* (partially digested intermediate molecules coated over the stomach wall, development of infection and inflammation) and thus, the person may have rapid peristaltic movements leading to diarrhoea.
- 4 When an individual continuously has the habit of consuming *Ushna* (hot potent) substances, that may lead to the situation called peristaltic rush. This occurs when there is infection in the small intestine condition such as infectious diarrhoea, there will be powerful and rapid peristalsis unlike normal weak and slow peristalsis. This is initiated by nervous reflexes that involve autonomic nervous system and brain stem and partly by intrinsic enhancement of myenteric reflex within the gut wall itself. Thus, the person may suffer from diarrhoea.
- 5 Normally, 6.5 to 7 litres of secretions per day is secreted by glands present in the alimentary canal. Another 1.5 litres of fluid will be taken through oral cavity by either water and other fluids. When the fluid intake exceeds 1.5 litres, this may lead to rapid movement of fluid in the alimentary canal leading to the excess fluid in the large intestine. Large intestine can absorb a maximum of 5 to 8 litres of fluid and electrolytes each day. When the fluid exceeds this amount, the person may have semiliquid stool passage.
- 6 Here, the substances which are large in size, usually will have heavier weight and they will move slow in the alimentary canal. The mechanism of pathological process will be similar to the mechanism seen in the consumption of *Guru* substances.

- ❖ Excess consumption of *Shita*⁷ (cold in touch and potency) substances
- ❖ Excess consumption of *Viruddha*⁸ (unwholesome) food substances
- ❖ Consumption of food substances before the digestion of previously ingested meal
- ❖ Consumption of food substances in a state of **indigestion**⁹
- ❖ Consumption of food substances at improper quantity and improper time
- ❖ Excess administration of *Snehana* (oleation) etc therapy
- ❖ Improper administration of *Snehana etc therapy*¹⁰
- ❖ Consumption of **toxic substances**¹¹
- ❖ Fear
- ❖ **Grief**¹²
- ❖ Consumption of **contaminated water**¹³
- ❖ Excess **alcohol consumption**¹⁴
- ❖ Consumption of **unwholesome food substances**¹⁵
- ❖ Consumption of food substances which are not grown in that particular **season**¹⁶

- ❖ **Swimming**¹⁷
- ❖ Suppression of natural urges
- ❖ Presence of **worm infestation**¹⁸

Samprapti of Atisara

संशम्यापधातुरनिं प्रवृद्धः शकुन्मिश्र वायुनाऽधः प्रणुनः।
सरत्यतीवातिसारं तमाहुर्व्याधिं घोरं षड्विधं तं वदन्ति॥ Ma.Ni. - 3/4

- ❖ Due to above said etiological factors, there will be vitiation of *Vata Dosha*, *Apana Vata* in particular
- ❖ There will be suppression of *Jatharagni* due to the excess fluid portion in the *Mahasrotas*.
- ❖ Excess water portion will move downwards and mixes with faecal matter resulting in loose stools and if not managed appropriately, then it will be a life threatening illness.

Samprapti Ghataka

- ❖ **Dosha** : *Vata*
- ❖ **Dushya** : *Purisha, Rasa*
- ❖ **Srotas** : *Purishavaha Srotas, Rasavaha Srotas*
- ❖ **Srotodushti Prakara** : *Atipravritti*

- 7 The consumption of *Shita* (cold) substances will lead to vasoconstriction of alimentary canal leading to decreased secretion of digestive juices right from the oral cavity to intestines. This will lead to the macromolecules which are present in the stomach may remain for longer time and thereby the person may develop infection and inflammation of alimentary canal, thus the person may develop diarrhoea.
- 8 Frequent consumption of *Viruddha* substances may be responsible for the inflammation of alimentary canal and thus the person may have rapid peristaltic movements and thus the person may develop diarrhoea.
- 9 When an individual has this type of habit, there will be
 1. Newly consumed substances will be present in the centre of the stomach which are more heavier in weight than previously consumed substances.
 2. Already consumed substances which are intermediate molecules will move to the periphery of the stomach.
 3. Normally, when an individual consumes food, pyloric antrum will close for 2 hours. As the person has taken the substances before the digestion of previously ingested meal, there will be closure of pyloric antrum for another 2 hours. This means the pyloric antrum will close more than 2 hours.
 4. Intermediate molecules which are situated in the wall of the stomach, may lead to infection in the alimentary canal and this in turn will lead to the inflammation of stomach.
 5. Infective and inflammatory diarrhoea will lead to increased peristaltic movements in the small intestine, thereby protecting the mucosa of the small intestine and this will lead to the increased peristaltic movements in the large intestine and thus leading to diarrhoea.
- 10 In this type of aetiological factor, the mechanism of pathological events are similar to excess consumption of *Atisnigdha* (unctuous substances).
- 11 This will lead to the inflammation of the alimentary canal and thus leading to increased peristaltic movements in the stomach, small intestine and large intestine and thus leads to the manifestation of diarrhoea.
- 12 This mechanism is called as psychogenic diarrhoea. This is caused by excessive stimulation of parasympathetic nervous system which greatly increases motility and excess secretion in the distal colon. Thus, the person may develop diarrhoea.
- 13 Consumption of contaminated water will lead to the infection and inflammation of alimentary canal and this will lead to the increased, rapid and powerful peristaltic movements and thereby the person may have diarrhoea.
- 14 Chronic excess alcoholism may lead to either alcoholic gastritis or alcoholic liver disease. When there is alcoholic gastritis, there will be increased, rapid and powerful peristaltic movements in the stomach, small and large intestines leading to the manifestation of diarrhoea. When there is alcoholic liver disease, the person may have portal hypertension which in turn leads to the congestion in the stomach, small and large intestines, thereby the person may develop diarrhoea.
- 15 This will lead to the similar mechanism of pathological events seen in the consumption of *Viruddha Ahara*.
- 16 When the person is having to exposure to this type of *Nidana*, the person may show the pathological events similar to the events occur in case of exposure to *Viruddha Ahara* consumption.
- 17 Excessive swimming may lead to increased peristaltic movements and thereby the person may develop diarrhoea. Another probable hypothesis could be the person may have psychogenic diarrhoea.
- 18 This is especially seen in children, specially seen in intestinal parasites or worm infestation. These are associated with uneasy abdominal sensation, night terrors, picking of the nose, itching of the perianal region and vulvo vaginitis

- ✱ Agni : Jatharagnimandya
- ✱ Ama : Jatharagnimandyajanya
- ✱ Udbhava Sthana : Pakvashaya
- ✱ Sanchara Sthana : Sarva Sharira
- ✱ Vyakta Sthana : Guda
- ✱ Rogamarga : Abhyantara and Bahya
- ✱ Sadhyasadyata : Sadhya to Asadya depending upon the duration of illness, severity of symptoms

Purvarupa of Atisara

हुनामिपायूदरकुक्षितोद्भावसानिलसन्निरोधाः।

बिद्सङ्गा आध्मानमथाविपाको भविष्यतस्तस्य पुरः सराणि॥5॥

Ma.Ni. – 3/5

- ✱ Pricking type of pain in the chest region
- ✱ Pricking type of pain in the umbilical region
- ✱ Pricking type of pain in anal region
- ✱ Pricking type of pain in the abdomen
- ✱ Pricking type of pain in the epigastric region¹⁹
- ✱ Sluggish movements of the body parts²⁰
- ✱ Stoppage of movement of Vata in the abdomen
- ✱ Constipation
- ✱ Distension of abdomen
- ✱ Impaired digestion²¹

Sankhya Samprapti of Atisara

6 types as per Charaka

1. Vataja Atisara
2. Pittaja Atisara
3. Kaphaja Atisara
4. Sannipataja Atisara
5. Bhayaja Atisara
6. Shokaja Atisara

6 types as per Sushruta and Madhavakara

1. Vataja Atisara

2. Pittaja Atisara
3. Kaphaja Atisara
4. Sannipataja Atisara
5. Shokaja Atisara
6. Amaja Atisara

Vataja Atisara

अथावरकालं वातलस्य वातातपव्यायामातिमा।

त्रनिषेविणो रुक्षाल्पप्रमिताशिनस्तीक्ष्णमद्यव्यवायनित्यस्योदावर्तयतश्च वेगान् वायुः प्रकोपमापद्यते, पक्का चोपहन्यते स वायुः कुपितोऽद्वाबुपहते मूत्र- स्वेदौ पुरीषाशयमुपहत्य, ताभ्यां पुरीषं द्रवीकृत्य, अतीसाराय प्रकल्पते।

तस्य रूपाणि - विज्जलमामं विप्लुतमवसादि रुक्षं द्रवं सशूल मामगन्धमीषच्छ ब्दमशब्दं वा विबद्धसूत्रवातमतिसार्यते पुरीषं वायुश्चान्तःकोष्ठे सशब्दशूलस्तिर्यक् चरति विबद्ध इत्यांमातिसारो वातात्।

पक्कं वा विवदमल्याल्प सशब्दं सशूलफेनपिच्छापरिकर्तकं दृष्टरोमा विनिःश्वसन् शुष्कमुखः कटयूरुत्रिकजानुपृष्ठपार्श्व शूली भ्रष्टगुदो मुहुर्मुहुर्विग्रथितमुपवेश्यते पुरीषं वातात्।

तमादुरनुग्रथितमित्येके, वातानुग्रथितवस्त्वात्॥5॥

Ca.Ci. – 19/5

अरुण फेनिलं रुक्षमल्पमल्पं मुहुर्मुहुः।

शकृदामं सरुक्शब्दं मारुतेनातिसार्यते॥6॥

Ma.Ni. – 3/6

- ✱ When a Vata Prakriti individual is exposed to wind, sun and physical exercise excessively
- ✱ Excess consumption of Ruksha (dry) food substances
- ✱ Frequently consuming little amount of food
- ✱ Consumption of food substances after several time after the appearance of hunger
- ✱ Excess consumption of Tikshna (irritating) substances such as chillies etc
- ✱ Excess consumption of alcoholic beverages
- ✱ Excess indulgence in sexual intercourse
- ✱ Suppression of natural urges frequently
- ✱ By above factors, there will be vitiation of Vata Dosha and function of Agni is hampered. This vitiated Vata will move Mutra and Sveda to the Pakvashaya leading

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- 19 These clinical features may indicate the ongoing the inflammatory changes happening in the oesophagus, stomach, small intestine, large intestine, rectum and anus.
- 20 This is attributable to hyponatremia that may occur as a result of loss of intestinal secretions through the anus to the exterior. This may become lethal within few hours.
- 21 When an individual is presented with Purvarupa of Atisara, firstly there will be stoppage of movement of Samana and Apana Vata, both of which may lead to the reduced movement of Vata in the Koshtha resulting in the manifestation of distension of the abdomen secondary to the hampered movement of Vata. As there is affliction of Samana Vata, there will be derangement of Ahara Parinamakara Bhava. The outcome will be impaired digestion.
When an individual consumes food substances which gets stored in the stomach for almost 2 hours. These consumed substances are in the macromolecular form and these substances are heavier in nature. These heavier substances will be moving in the small intestine, lying closer to the intestinal wall, leading to the fullness of the intestine. As these substances reach the large intestine with much delay, the large intestine will be absorbing the water from the rectum, sigmoid and descending colon, leading to the formation of hard stools and this will lead to constipation.
As the person is having more amount of macromolecules in the stomach, small and large intestines and these lie closer to the intestinal wall, secretions from the small and large intestine will not reach the intestine leading to the indigestion state.

to the manifestation of *Vataja Atisara*.

Lakshana of Vataja Atisara

- In case of *Ama* dominant *Vataja Atisara*, the faecal matter will sink in the water, faecal matter spreads quickly, rough, liquid consistency, painful defaecation with fishy odour, passes with little sound or no sound during defaecation associated with retention of urine and flatus. The retained *Vayu* moves within the abdomen with sound and pain.
- In case of *Pakva Vataja Atisara*, the person will pass hard stool in little quantity with sound and pain. The faecal matter contains froth, slimy in texture, cutting type of pain around anus, horripilation, increased respiration, dryness of mouth, pricking type of in waist, thigh, sacrum, knee, back and sides of the abdomen. The person may have prolapse of rectum. The frequency of passage of liquid stools is higher than all other types of *Atisara* and the nature of faecal matter is unformed.

Pittaja Atisara

पित्तलस्य पुनरुल्लवणकटुकक्षारोष्णातीक्ष्णातिमात्रनि-
प्रतताडिसूर्यसंतापोष्णमारतोपहतगात्रस्य क्रोधेर्ष्याबद्दलस्य पित्तं
प्रकोपमापद्यते।
तत् प्रकुपितं द्रवत्वाद्दूष्माणमुपहत्य पुरीषाशय- विसृतमौण्य्याद्
द्रवत्वात् सरत्वाच्च भित्त्वा पुरीष- मतिसाराय प्रकल्पते।
तस्य रूपाणि-हारिद्रं हरितं नीलं कृष्णं रकेपित्तोपहितमति
दुर्गन्धमतिसार्यते पुरीषं, तृष्णादाहस्वेदमूर्च्छाशूलननसंतापपाकपरीत
इति पित्तातिसारः॥६॥
पित्तात्पीतं नीलमालोहितं वा तृष्णामूर्च्छादाहपाकोपपन्नम्।

Ca.Ci. - 19/6

Ma.Ni. - 3/7

- Excess intake of *Amla* (sour), *Lavana* (salty) and *Katu* (pungent) *Rasa* substances
- Excess intake of *Kshara* (alkaline) substances
- Excess consumption of *Ushna* (hot in potency) and *Tikshna* (irritating) substances
- Excess exposure to fire e.g. housewives and cooks
- Excess exposure to sunlight e.g. labour workers
- Excess exposure to *Ushna Maruta* (hot air)
- Anger
- Jealousy

Due to increase of *Dravatra* (liquid) in the body, there will be decrease in digestive fire. This decreased fire will enter the *Pakvashaya*, due to its *Ushnatva*, *Dravatra* and *Saratva*, *Pittaja Atisara* will be manifested in the individual.

Lakshana of Pittaja Atisara

- Yellowish, greenish, bluish or blackish discolouration of liquid or semiliquid faecal matter or blood mixed

liquid or semiliquid faecal matter with foul smell

- Thirst
- Burning sensation
- Excess sweating
- Altered state of consciousness
- Pricking type of pain in the abdomen
- Rectal prolapse
- Raised body temperature
- Suppuration

Kaphaja Atisara

श्लेष्मलस्य तु गुरुमधुरशीत स्निग्धोपसेविनः संपूरकस्याचिन्तयते
दिवाखमपरस्यालसस्य लेष्मा प्रकोपमापद्यते।
स स्वभावाद् गुरुमधुरशीतस्निग्धः स्वस्तोऽग्निमुपहत्य सौम्यस्वभावात्
पुरीषाशयमुपह- त्योपक्लेद्य पुरीषमतिसाराय कल्पते।
तस्य रूपाणिस्निग्धं श्वेतं पिच्छिलं तन्तुमदामं गुरु दुर्गन्धं लेमोप-
हितमनुबद्धशूलमल्पाल्पमभीक्ष्णमतिसार्यते सप्रवाहिकं,
गुरुदरगुदवस्तिबंधक्षणेदेशः कृतेऽप्य- कृतसंज्ञः सलोमहर्षः सोत्क्लेशो
निद्रालस्यपरीतः सदनोऽन्नद्वेषी चेति श्लेष्मातिसारः॥७॥

Ca.Ci. - 19/7

शुक्लं सान्द्रं श्लेष्मणा श्लेष्मयुक्तं विलं शीतं दृष्टरोमा मनुष्यः॥
Ma.Ni. - 3/7

- In a *Kapha Prakriti* individual, if the person consumes *Guru* (heavy to digest) substances
- Excess consumption of *Madhura* (sweet) *Rasa* substances
- Excess intake of *Shita* (cold in touch and potency) substances
- Excess intake of *Snigdha* (unctuous) substances
- Excess intake of food in large quantity at a time
- The person who has less mental stressful activities
- The person who indulges in sleeping in day time
- Lazy person

Due to the *Guru*, *Madhura*, *Shita* and *Snigdha* qualities which are opposite to *Agni* (digestive fire) by nature, the digestive fire will move to *Pakvashaya*, leading to the manifestation of *Kaphaja Atisara*.

Lakshana of Kaphaja Atisara

- Faecal matter will be unctuous, whitish discolouration, slimy. The faecal matter will be thready in nature. It will sink in water, having foul smell, with the presence of mucus in the faecal matter.
- Pain in the abdomen
- Frequent passage of liquid or semiliquid stools
- Tenesmus
- Heaviness of the abdomen

- ⊗ Heaviness of the lower abdomen
- ⊗ Heaviness of the groin region
- ⊗ The person will have the feeling of urge of defaecation even after passage of stools
- ⊗ Horripilation
- ⊗ Nausea
- ⊗ Excess sleep
- ⊗ Laziness
- ⊗ Sluggish movement of body parts
- ⊗ Aversion to take food

Sannipataja Atisara

अतिशीतस्निग्धरुक्षोष्णगुरुखरकठिनविषमविरुद्धासात्म्य भोजनाद-
भोजनात् कालातीतभोजनाद् यत्किंचिदभ्यवहरणात् प्रदुष्टमद्यपानीय-
पानादतिमद्यपानादसंशोधनात् प्रतिकर्मणां विषमगमनादनुपचाराज्ज्वल-
नादित्यपवन सलिलाति से बनादस्वप्नादतिखनाद्वेगविधारणातुवि-
पर्ययादयथाबलमारम्भाद्भयशोकचित्तोद्वेगातियोगात् कृमिशोषज्वरा-
शौविकारातिकर्षणाद्वा व्यापन्नानेत्रयो दोषाः प्रकुपिता भूय
एवाग्निमुपहत्य पक्वाशयमनुप्रविश्यातीसारं सर्वदोषलिङ्गं जनयन्ति॥८॥

Ca.Ci. – 19/8

- ⊗ Excess intake of cold substances
- ⊗ Excess intake of unctuous substances
- ⊗ Excess intake of dry substances
- ⊗ Excess intake of substances having hot potency
- ⊗ Excess intake of substances which are heavy to digest
- ⊗ Excess intake of substances which are rough
- ⊗ Frequent consumption of food substances at improper quantity and time
- ⊗ Excess intake of unwholesome food substances
- ⊗ Excess intake of incompatible food substances
- ⊗ Frequent fasting
- ⊗ Consumption of food substances after several hours of appearance of hunger
- ⊗ Consumption of little amount of food
- ⊗ Consumption of contaminated water, contaminated alcoholic beverages
- ⊗ Excess intake of alcoholic beverages
- ⊗ Improper Shodhana (purification or Panchakarma) therapy
- ⊗ Improper administration of treatment
- ⊗ Improper Agni Karma treatment
- ⊗ Improper exposure to sun
- ⊗ Excess intake of water
- ⊗ Avoiding sleep or excess sleep

- ⊗ Suppression of natural urges
- ⊗ Not following appropriate measures in different seasons
- ⊗ Doing activities which cause physical exertion which are not in accordance with one's strength
- ⊗ Excess mental stress such as fear, grief and anxiety
- ⊗ Krimi (Worm infestation)
- ⊗ Shosha (Rajayakshma where excess depletion of all Dhātu is seen)
- ⊗ Jvara (fever)
- ⊗ Arshas (hemorrhoids)
- ⊗ Excess activities which lead to emaciation

All vitiated *Dosha* will reduce *Agni* (digestive fire). This *Agni* will move to *Pakvashaya* leading to the manifestation of *Atisara*.

Krichrasadhya Sannipataja Atisara Lakshana

अपि च शोणितादीन् धातूनतिप्रकृष्टं दूषयन्तो धातुदोषस्वभावकृतान-
तीसारवर्णानुपदर्शयन्ति।

तत्र शोणितादिषु धातुष्यतिप्रदुष्टेषु हारिद्रहरितनीलमा- जिष्ठ
मांसधावनसन्निकाशं रक्तं कृष्णं श्वेतं वराहभेदः सद्दृशमनुबद्धवेदनमवेदनं
वा समासव्यत्या सादुपवेश्यते शकृद् प्रथितमामं सकृत् सकृदपि
पक्कमनतिक्षीणमांसशोणितबलो मन्दाग्निर्विहतमु खरसश्च;
तादृशमातुरं कृच्छ्रसाध्यं विद्यात्।

एभि- वर्णैरतिसार्यमाणं सोपद्रवमातुरमसाध्योऽयमिति प्रत्याचक्षीत
तद्यथा- पक्कशोणितानं यकृत्खण्डो- पर्म मेदोमांसोदकसन्निकाशं
दधिघृतमज्जतैलवसा- क्षीरबेसवाराभमतिनीलम तिरक
मतिकृष्णमुदक- मिवाच्छं पुनर्मैचकाभमतिस्निग्धं हरितनीलकषाय-
वर्णं कर्बुरमाविलं पिच्छिलं तन्तुमदामं चन्द्रकोप- गतमति
कुणप पूतिपूयगन्ध्यामाममत्स्यगन्धि मेक्षिकुथितबहुधातुस्त्राव
मल्पपुरीषमपुरीषं वाऽतिसार्यमाणं तृष्णादाहज्वरभ्रमतम कहि
काश्यासानुबन्धमतिवेदनमवेदनं वा त्रस्तपकगुदं पतितगुदवलि
मुक्तनालमतिक्षीणवलमांसशोणितं सर्वं पर्वोस्थिशूलिनमरोचकारतिप्रल
पसंमोहपरीतं सहसोपरत विकारमतिसारिणमचिकित्स्यं विद्यात्।

इति सन्निपातातिसारः॥९॥

Ca.Ci. – 19/9

वराहस्नेहमांसाम्बुसद्दृशं सर्वरूपिणम्।

कृच्छ्रसाध्यमतीसारं विद्यादोषत्रयोद्भवम्॥८॥

Ma.Ni. – 3/8

- ⊗ Yellowish, greenish, bluish, slight reddish, reddish, blackish, whitish discolouration of faecal matter; faecal matter having the colour of water which is used to clean meat; faecal matter having the colour of fat of pig
- ⊗ May or may not be associated with pain
- ⊗ Frequent passage of unformed and formed stools
- ⊗ Less muscle atrophy and less loss of blood in the body and mild loss of physical strength
- ⊗ Less digestive fire

Asadhya Sannipataja Atisara Lakshana²²

- The stool resembling the colour of cooked blood
- The stool having the colour of liver pieces
- The stool having the colour of meat soup or fat
- The stool having the consistency of curds, ghee, bone marrow, oil, muscle fat, milk, *Vesantara* (a spicy preparation)
- The stool having the colour of dark blue, dark red, dark black or water like clear appearance
- The stool having the colour of *Mechaka* (eye of a peacock's tail)
- Too unctuous faecal matter
- The faecal matter having green, blue or orange colour
- The stool which has *Karbura* appearance (variegated colour or streaked or spotted appearance)
- Turbid appearance of stool
- Slimy appearance of stool
- Thready appearance of stool
- Stool with moony lustre
- Stool with foetid, putrid and cadaverous odour, odour of uncooked fish
- Stool attacked by flies
- Stool containing profused discharge of necrosed tissues
- Stools containing multiple *Dhatu* in it
- Passage of little or no faecal matter
- Thirst
- Burning sensation
- Fever
- Giddiness
- Darkness in front of the eyes
- Hiccups
- Breathing discomfort
- Associated with severe pain or no pain at all
- Prolapse of rectum or suppuration at rectum
- Too physically weak individual
- Too muscle atrophy
- Too little blood in the body
- Multiple joint pain and pain in bones
- Tastelessness
- Irrelevant and incomprehensible speech
- Confusion

Bhayaja and Shokaja Atisara

आगन्तु द्वावतीसरी मानसी भयशोकजी।
तत्तयोर्लक्षणं वायोर्यदतीसारलक्षणम्॥11॥

Ca.Ci. - 19/11

- As per *Charaka*, *Bhayaja* and *Shokaja*²³ *Atisara* are *Agantuja* as far as the cause of *Atisara* is concerned.
- These are considered as *Manasika Vikara*.
- The *Lakshana* of these types of *Atisara* are similar to *Vataja Atisara*.

Shokaja Atisara

तैस्तैर्भविः शोचतोऽल्पाशनस्य वाष्पोष्मा वै वहिमाविश्य जन्तोः।
कोष्ठं गत्वा क्षोभयेत्तस्य रक्तं तच्चाधस्तात्काकणन्ती-प्रकाशम्॥9॥
निर्गच्छेद्वै विविमिश्रं ह्यविद्धा निर्गन्धं वा गन्धवद्वाऽतिसारः।
शोकोत्पन्नो दुश्चिकित्स्योऽतिमात्रं रोगो वैद्यैः कष्ट एषप्रदिष्टः॥10॥

Ma.Ni. - 3/9,10

- The person who is having serious mental stress and who consumes very little amount of food or consumes food with grief, vitiated *Dosha* will reach the abdomen and decrease the *Agni* of the individual.
- Person will pass the stools mixed with blood which simulates the colour of *Gunja*.
- The person passes the faeces mixed with blood associated with or without foul smell, passes with difficulty.
- This type of *Atisara* is very difficult to manage in most of the occasions and has been treated as major trouble by most of the clinicians.

Amaja Atisara

अन्नाजीर्णात्प्रदुताः क्षोभयन्तः कोष्ठं दोषा धातुसंघान्मलांश्च।
शूलोपेतं षष्ठमेनं वदन्ति॥11॥

Ma.Ni. - 3/11

- *Dosha* excited by the presence of *Ama* in the body leads to the vitiation of *Dosha*. These vitiated *Dosha* will enter the *Koshtha* (alimentary canal).
- The person will pass the faecal matter having multiple colours and all these different colours are not seen in one frequency of bowel movement.
- This is associated with severe abdominal pain.

Raktaja Atisara

पित्तकृन्ति यदाऽत्यर्थं ब्रव्याण्यश्नाति पैत्तिके।

तदोपजायते ऽभीक्ष्णं रक्तातीसार उल्बणः॥20॥

Ma.Ni. - 3/20

The main feature of this type of *Atisara* is the appearance of the blood in the liquid or semisolid stools.

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22 Refer the *Asadhya Lakshana* of *Atisara* described later in the chapter

23 The terms *Bhaya* (fear) and *Shoka* (grief) are considered as emotional disturbances. As per western physiology, there will be extreme activation of parasympathetic nervous system in case of extreme emotional disturbances. Extreme activation of parasympathetic nervous system is responsible for excess secretion of mucus in the large intestine by the mucous cells of the large intestine. The frequency will be increased as rapid as every 30 minutes.

- ⊛ The causes of this type of *Atisara* is excess intake of the substances *Pitta* vitiating diet for a longer period of time.

Samatva and Niramatva of Atisara

संसृष्टमेभिर्दोषैस्तु न्यस्तमप्स्ववसीदति।
पुरीषं भृशदुर्गन्धि पिच्छिलं चामसंशितम्॥1 2॥
एतान्येव तु लिङ्गानि विपरीतानि यस्य वै।
लाघवं च विशेषेण तस्य पकं विनिर्दिशेत्॥1 3॥

Ma.Ni. – 3/12,13

Sama Atisara Lakshana

- ⊛ When the vitiated *Dosha* mixed with faecal matter associated with *Ama*²⁴ will have following features.
- ⊛ The faecal matter will sink in water, indicates the *Gurutva*²⁵ of the faecal matter
- ⊛ Very foul smelling²⁶
- ⊛ Slimy nature²⁷

Nirama Atisara Lakshana

- ⊛ The faecal matter will float in the water²⁸.
- ⊛ The faecal matter will not have foul smell and non slimy in nature²⁹.

Asadhya Lakshana of Atisara

पक्वजाम्बवसंकाशं यकृतखण्डनिभं तनु।
घृततैलबसामज्जवेशाबापरयोदधि॥1 4॥
मांसधावनतोयानं कृष्णं नीलारुणप्रभम्।
मेचकं सिग्धकपूरं चन्द्रकोपगतं धनम्॥1 5॥
कुणपं मस्तुलुङ्गाभं सुगन्धि कुथितं बहु।
तृष्णादाहतमः श्वासहिकापावस्थिशूलिनम्॥1 6॥
समूच्छरितिसंमोहयुक्तं पचबलीगुदम्।
प्रलापयुक्तं च भिषग्बर्जयेदतिसारिणम्॥1 7॥
असंवृतगुदं क्षीणं दूराध्मातमुपडुतम्।
गुदे पक्वे गतोष्माणमतिसारकिणं त्यजेत्॥1 8॥
श्वासशूलपिपासात क्षीणं ज्वरनिपीडितम्।
विशेषेण न वृद्धमतीसारो विनाशयेत्॥1 9॥

Ma.Ni. – 3/14-19

- ⊛ The colour of faecal matter resembles the colour of ripened *Jambu Phala*³⁰
- ⊛ The colour resembles the colour of piece of liver²⁸
- ⊛ The consistency of faecal matter is thin³¹
- ⊛ The faecal matter resembles the ghee, oil, muscle fat, bone marrow, *Veshavara* (a spicy preparation prepared with meat), milk or curds, or colour of water used to wash meat, blackish discolouration, bluish or reddish discolouration³².

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- This is usually associated with the presence of macromolecules in the faecal matter due to the hampered alimentary canal secretions.
- When the ingested food substances which are macromolecular in nature will enter into the small intestine without proper digestion i.e. they will enter in the macromolecular form. Even in the small and large intestines, these macromolecules will move to the exterior in the same fashion owing to lack of secretions in the alimentary canal. These are heavier than the water present in the exterior. Thus, in case of *Sama Atisara*, we can find the sinking of faecal matter in the lavatory. This indicates *Samatva* of *Purisha*.
- This is indicative of the presence of excess gas formation (methane and hydrogen sulphide in particular) in the large intestine due to the action of colonic bacteria on the unabsorbed carbohydrates in the large intestine. Certain food substances such as beans, cabbage, onion, cauliflower, corn and some irritants such as vinegar which act as suitable medium for the increased gas formation in the large intestine. In most of the occasions, excess expulsion of gases results from rapid peristaltic movement in the large intestine.
- Slimy nature of the faecal matter is attributable to the presence of excess mucus in the large intestine. Large intestine contains no villi like small intestine and epithelial cells contain no enzymes, hence no digestion takes place in the large intestine, only absorption of micronutrients such as electrolytes, water may occur in the large intestine. Consumption of any irritating substances if enters into the large intestine, it may lead to excess mucus secretion and the faecal matter will have more slimy consistency.
- Normally, when a substance enters ileocaecal junction, it may take one to two days for its travel from caecum to sigmoid colon. Nearly about 1200 ml of fluid enters the caecum and 100 ml of fluid portion will move further to the rectum. This indicates the absorption of > 90% of water takes place normally. There will be absence of macromolecules which enter into the large intestine indicating the complete digestion and assimilation of micronutrients happening in the small intestine. Hence the person will have passage of lighter faecal matter leading to the floating of the faecal matter in the water.
- When there is proper digestion and assimilation of carbohydrates is occurring, colonic bacteria will get less carbohydrates for the production of methane and hydrogen sulphide. Hence, the person will not have foul smell in the faecal matter during defaecation. Non slimy nature of faecal matter suggests the reduced mucus secretion owing to less irritation of digested substances entering into the large intestine.
- The discolouration explained in this context is dark blue or blackish discolouration. When an individual is having diarrhoea with these clinical features, it may indicate the bleeding occurring in the stomach. Because of the formation of acid haematin in the stomach due to the mixture of hydrochloric acid with blood in the stomach gives the dark blue or blackish discolouration to the faecal matter. This may indicate the seriousness of the situation.
- Thin consistency of faecal matter indicates improper absorption of water from the large intestine. Normally, 1,000 to 1,200 ml of the chyme pass through ileocaecal junction each day and nearly about less than 100 ml will be expelled through anal orifice. This leads to the loss of sodium ions too along with water from the large intestine. 30 gms of sodium are secreted into the intestinal secretions by the body and nearly 5 to 8 gms of sodium is eaten by normal individual. To prevent the sodium loss from the body, the intestine has to absorb 25 to 35 gms of sodium each day. Thin consistency implies that the sodium along with water loss is inferred. If this situation is not corrected, within hours, the condition may be fatal.
- The colour resembling ghee, oil, muscle fat, bone marrow or *Veshavara* indicates the yellowish appearance of faecal matter. For example in case of steatorrhoea, the person may have fat indigestion.
 - The colour resembling milk or curds indicates the infection or inflammation occurring in the large intestine.

- The faecal matter resembles *Mechaka*³³ (eye of a peacock's tail).
- The faecal matter is unctuous.
- The stool which has *Karbura* appearance³⁴ (variegated colour or streaked or spotted appearance).
- The faecal matter resembles appearance of moon.
- The smell of faecal matter resembles foetid³⁵ smell or smell of brain or cadaverous smell³⁶.
- The condition is associated with thirst, burning sensation, darkness in front of the eyes, breathing discomfort, hiccups, pain in the sides of the chest and bones, altered state of consciousness, restlessness, suppuration in the rectum³⁷.
- If the person is having extreme loss of physical strength³⁸, distension of the abdomen³⁹, associated with *Upadrava*⁴⁰ (complication).
- If there is *Pakvata*⁴¹ (suppuration in rectum) associated with loss of appetite, breathing discomfort, pain

abdomen, thirst, extreme loss of physical strength, fever.

- Passage of faecal matter, flatus and urine without any sound⁴²
- Proper digestive fire⁴³
- Lightness of the alimentary canal⁴⁴

Atisara Mukta Lakshana

यस्योच्चारं विना मूत्रं सम्यग्वायुश्च गच्छति।
दीप्ताग्नेर्लघुकोष्ठस्य स्थितस्तस्योदरामयः॥२३॥

Ma.Ni. - 3/23

The following features indicate the restoration of the function of *Apana Vata* in the patient of *Atisara*.

Differential diagnosis of Atisara as per Madhukosha commentary:

एवं स्नेहाजीर्णविसूचिकाविषाणः क्रिमिप्रभृतिर

Madhu, Ma.Ni. - 3/20

Sneha Ajirna (appearance of liquid stools after the

- The colour resembling colour of water used to wash meat will be slight reddish discolouration. This may be observed in case of slight bleeding observed in the small or large intestine. In case of beetroot consumption too, the person may have reddish discolouration.
- Black or dark bluish discolouration may indicate either bleeding observed in the upper alimentary canal.

- 33 This colour is observed when there is presence of oil or fat in the faecal matter. This indicates the person is having the poor absorption in the small and large intestine.
- 34 Variegated colour indicates the presence of oil or unctuous substances in the faecal matter. This indicates steatorrhoea. Spotted appearance indicates the unformed stools. When an individual has continuous unformed stools indicates profound water and electrolytes loss which further suggests hypovolemia.
- 35 This is occurring due to the colonic bacterial action on the food residues entering the large intestine. These are responsible for the production of carbon dioxide, hydrogen and methane. These are formed depending upon the person's colonic bacterial flora and on the type of food consumed. Foetid smell or cadaveric smell is produced due to the excess formation of hydrogen sulphide.
- 36 This may indicate the presence of methane and hydrogen sulphide. This is attributable to the colonic bacterial action on undigested carbohydrates entering into the large intestine. This is indicative of inflammation which leads to irritation of the large intestine, and the person will have less time for the absorption of water and electrolytes from the large intestinal mucosa.
- 37 These symptoms can be attributable to either hypovolemia or electrolyte imbalances or abdominal muscle cramps etc.
- 38 When an individual is presented with *Atisara* along with extreme loss of physical strength, this is most commonly attributable to profound electrolyte loss occurring through diarrhoea, sodium ions loss in particular and profound water loss from the body. It is being said that normal sodium absorption will be around 25 to 35 gms per day. Normally, around 20 to 30 gms of sodium is secreted in the alimentary canal through different digestive secretions and around 5 to 8 gms of sodium is ingested through oral cavity. Around 0.5% of the intestinal sodium is lost in the faeces everyday. Sodium loss in diarrhoea can be lethal within hours because, for every skeletal muscular activity, smooth muscular activity, cardiac muscular activity, neurotransmission require adequate amount of sodium. In the absence of sodium, the person may have compromise in all these kinds of activities.
- 39 This can be attributable to either perforation or acute abdomen which may lead to paralytic ileus.
- 40 शोथं शूलं ज्वरं तृष्णां श्वासं कासमरोचकम्। छर्दिं मूर्च्छां च हिक्कां च दृष्ट्वाऽतीसारिणं त्यजेत्॥
Swelling, pain, fever, thirst, breathlessness, cough, tastelessness, vomiting, altered state of consciousness, hiccups are the *Upadrava* of *Atisara* as mentioned by Madhukosha commentary of Madhava Nidana in the context of *Atisara*
- 41 When an individual is having pus discharge through anus, it indicates either fistula in ano or anorectal abscess or carcinoma of rectum or ulcerative colitis. Most of the occasions, the person will avoid urge of defaecation owing to pain developed after defaecation.
- 42 When an individual presents with above said symptom indicates the proper absorption of water and gases. Normally, large intestine can absorb 5 to 8 litres of water. When one considers about the gas formation, 7 to 10 litres of gases are formed in the large intestine and only 0.6 litres of gases are expelled out through anus. The abundance in water and gases in the faecal matter suffering from diarrhoea is the reason for the production of sound during defaecation. When the individual is passing stools in the absence of sound indicates the absorption of water as well as gases from large intestine.
- 43 When an individual is presented with appearance of proper appetite at proper time indicates the restoration of function of alimentary canal i.e. reabsorption of fluid and gases from large intestine. This indirectly implies decrease of increased peristaltic movements in the alimentary canal. This mechanism also affects the proximal part of the alimentary canal including small intestine and stomach. This implies retention of water, electrolytes such as sodium and potassium too.
- 44 Lightness of the alimentary canal indicates the reabsorption of water and nutrients from the alimentary canal. It implies the proper transformation of macro molecules into micromolecules in the small intestine and large intestine. Both of these mechanisms are responsible for the appearance of feeling of the lightness of the alimentary tract by an ailing individual.

Madhu, Ma.Ni. - 3/14-19

consumption of *Sneha* (ghee or oil in the form of *Snehapana*); *Visuchika* (excess upward and downward movement of morbid *Dosha* associated with pricking type of pain all over the body); *Visha* (consumption of toxic substances); *Krimi* (presence of helminths in the body).

Each of the diseases can be differentiated from each other by identifying the presence or absence of associated features along with liquid stools.

Pravahika

Introduction

Pravahika (see Figure 9) is a spectrum of diseases related to *Mahasrotas*. Primary *Dosha* involved in the manifestation of the disease is vitiated *Vata*.

It is associated with the presence of *Kapha* (mucus) in the faecal matter. It is also termed as *Visramsī* by *Bhoja*, as *Antargranthi* by *Parashara* and *Nischaraka* by *Harita*. Large intestine is one such organ which produces large amount of mucus.

The peculiarity of large intestine is it contains many crypts of *Lieberkuhn* like small intestine, but it does not contain villi. The epithelial cells contain almost no enzymes.

This mucus contains moderate amounts of bicarbonate ions secreted by a few non-mucus secreting epithelial cells.

The rate of secretion of mucus is determined by direct, tactile stimulation of epithelial cells lining the large intestine and by local nervous reflexes.

During extreme stimulation of parasympathetic nervous system such as emotional disturbances, much amount of mucus is secreted by large intestine, so that increased movement of ropy mucus is passed through anus, but with no faecal matter. The time period of two movements of bowels is 30 minutes.

भोजादौ लियं विस्त्रंसीति नाम्ना पठ्यते,

पराशरे खन्तमेन्थि रिति, हारीतस्तु निश्चारकाख्यामेतां पठति।

Madhu, Ma.Ni. – 3/21

वायुः प्रवृद्धो निचितं बलासं नुदत्यधस्तादहिताशनस्य।

प्रवाहोऽल्पं बहुशो महाकं प्रवाहिकां तां प्रवदन्ति तज्ज्ञाः॥२१॥

Ma.Ni. – 3/21

- ✿ *Ahitashana*⁴⁵ (improper dietary pattern) is the main cause of the disease *Pravahika*.
- ✿ Vitiated *Vata* is the main cause for the manifestation of the disease.
- ✿ This vitiated *Vata* will be responsible for the downward movement of accumulated *Kapha*.
- ✿ The person will have less *Pravahana* (tenesmus)⁴⁶ and person will pass large amount of faecal matter mixed with mucus.

Samprapti Ghataka

- ✿ **Dosha** : *Vata* along with *Kapha*

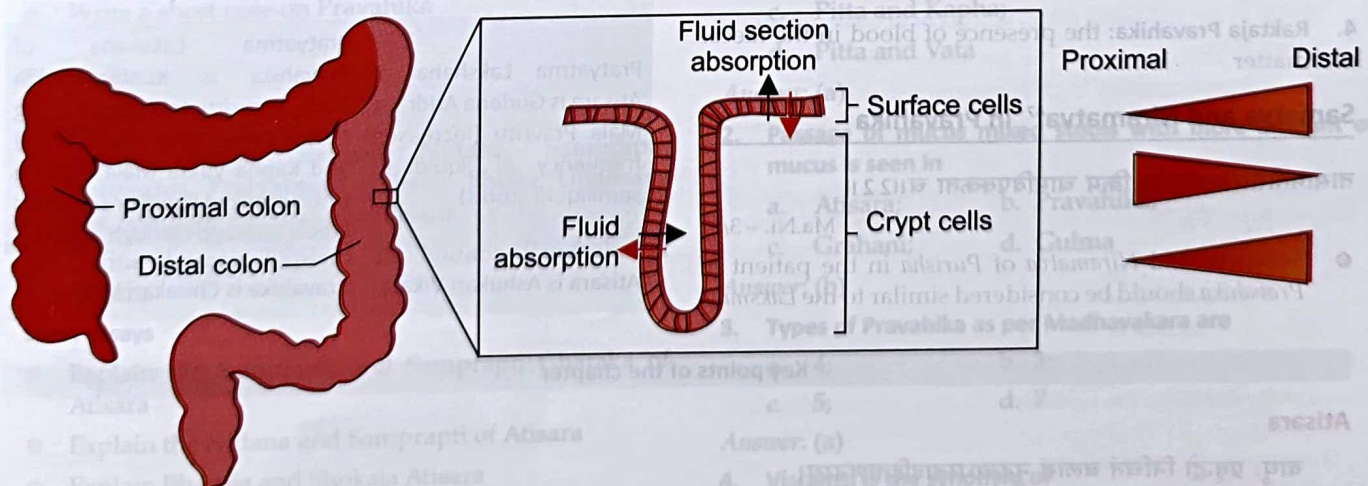


Figure 9: Pratyatma Lakshana and Samprapti of Pravahika

⁴⁵ The term improper diet pattern indicates the excess *Pitta* vitiating diet pattern. The substances which are having the qualities of *Ushna*, *Tikshna*, *Laghu* guna which increase the *Tejo* Mahabhuta is responsible for the vitiation of *Pitta* *Dosha* in the body, in the alimentary canal at the beginning.

Most of the acid containing the food substances are responsible for the increased peristaltic activity which in turn enters into the large intestine, leads to the excess secretion of mucus.

During extreme parasympathetic nervous stimulation such as emotional disturbances, much mucus is secreted and the person will have bowel movement of ropy mucus as often as every 30 minutes, which contains little or no faecal material.

⁴⁶ The term tenesmus refers to continual or recurrent inclination to evacuate the bowels. This symptom is usually caused by disorder of rectum.

- **Dushya:** Purisha, Rasa
- **Srotas:** Purishavaha and Rasavaha Srotas
- **Srotodushti Prakara:** Sanga and Vimargagamana
- **Agni:** Jatharagnimandya
- **Ama:** Jatharagnimandyaajanya Ama
- **Udbhava Sthana:** Pakvashaya
- **Sanchara Sthana:** Sarva Sharaira (as Rasa Dhatu affliction is seen)
- **Vyakta Sthana:** Guda
- **Rogamarga:** Abhyantara and Bahya
- **Vyadhi Svabhava:** Ashukari
- **Sadhyasadhyata:** Sadhya to Krichrasadhyata depending upon the duration and severity of symptoms

Types of Pravahika

प्रवाहिका वातकृता सशूला पित्तात्सदाहा सकफा कफाच।
सशोणिता शोणितसंभवा च ताः स्नेहलक्षप्रभवा मतास्तु।

Ma.Ni. – 3/21,22

4 types of Pravahika:

1. **Vataja Pravahika:** pricking type of pain
2. **Pittaja Pravahika:** burning sensation during defaecation
3. **Kaphaja Pravahika:** presence of mucus in the faecal matter
4. **Raktaja Pravahika:** the presence of blood in the faecal matter

Samatva and Niramatva⁴⁷ in Pravahika

तासामतीसारवदादिशेष क्रिम चामविपक्वता च॥22॥

Ma.Ni. – 3/22

- **Samatva and Niramatva of Purisha** in the patient of **Pravahika** should be considered similar to the **Laksnana**

mentioned in the context of **Samatva** and **Niramatva** of **Purisha** in the patient of **Atisara**.

संयुष्टमेभिर्दोषैस्तु न्यस्तमप्यवसीदति।
पुरीषं भृशदुर्गन्धि पिच्छलं चामसंज्ञितम्॥1 2॥

एतान्येव तु लिङ्गानि विपरीतानि यस्य वै।
लाघवं च विशेषेण तस्य पक्वं विनिर्दिशेत्॥1 3॥

Ma.Ni. – 3/12,13

- **Samatva of Purisha in Pravahika:** the person will have unformed stools which will sink in water when we put the faecal matter to the water, foul smell, slimy consistency
- **Niramatva of Purisha in Pravahika:** the person will not be suffering from unformed stools and that will not sink in water, in stead, it will float in the water. The faecal matter will not have foul smell and consistency will not be slimy.

Difference between Atisara and Pravahika:

Atisara	Pravahika
Atisara primarily involves Purishavaha Srotas followed by Rasavaha Srotas affliction	Pravahika primarily involves Purishavaha Srotas
Short duration of the illness is the hallmark of this disease	Long duration of the illness is the hallmark of this disease
Pratyatma Lakshana of Atisara is Gudena Atidrava Mala Pravritti (increased frequency of liquid to semiliquid stools)	Pratyatma Lakshana of Pravahika is Kunthanayukta Mala Pravritti (excess straining during defaecation or tenesmus) and Kapha yukta Mala Pravritti (mucus mixed liquid or semisolid stools passage through anus)
Atisara is Ashukari Vikara	Pravahika is Chirakari Vikara

Key points of the chapter

Atisara

वायुः प्रवृद्धो निचितं बलासं नुदत्यधस्तादहिताशनस्य।

प्रवाहतोऽल्पं बहुशो मलाक्कं प्रवाहिकां तां प्रवदन्ति तज्ज्ञाः॥29॥ Ma.Ni. – 3/21

- Vata and Kapha Dosha are the main initiating factors for the manifestation of the Pravahika.
- The main cause of Pravahika is Ahitashana (improper diet pattern)
- In this disease, the person will have more strain for defaecation and the amount of faecal matter formed is less but more amount of mucus is expelled to the exterior.
- As far as Sankhya Samprapti of Pravahika is concerned, 4 types of Pravahika are mentioned – Vataja, Pittaja, Kaphaja, Raktaja

Pravahika

गुदेन बहुद्रव- सरणमतिसार इत्यर्थः।

Ma.Ni. – 3/4, Madhu

- ❖ Excess amount of passage of either liquid or semisolid stools through anal orifice is termed as Atisara.
- ❖ As far as the Ayurveda fundamentals, there will be initially Agnimandya followed by movement of excess liquid substances to the Pakvashaya leading to Apana Vata vitiation resulting in the manifestation of Atisara.
- ❖ As far as Sankhya Samprapti of Atisara is concerned, it is of 6 types. As per Charaka, 6 types of Atisara are Vataja, Pittaja, Kaphaja, Sannipataja, Bhayaja and Shokaja, where as Madhavakara has mentioned 6 types of Atisara are Vataja, Pittaja, Kaphaja, Sannipataja, Shokaja, Amaja.
- ❖ Vit Sanga (constipation) is a specific type of Purvarupa observed in this disease which will be absent in the full blown manifestation of Atisara.
- ❖ When an individual is presented with excess diarrhoea, the person may have profound loss of water from the body leading to hypovolemia, electrolyte imbalances leading to extreme fatigue.

Probable questions asked in the university examination

Atisara**Long essay**

- ❖ Long essay question from this chapter point is rare.

Short essays

- ❖ Explain Pravahika
- ❖ Explain the Nidana and Samprapti of Pravahika
- ❖ Explain Samprapti and Samprapti Ghataka of Pravahika

Short answers

- ❖ Define Pravahika
- ❖ Enumerate the types of Pravahika
- ❖ Write a short note on Pravahika

Pravahika**Long essay**

- ❖ Define Atisara. Explain Nidana, Samprapti, Purvarupa, Pratyatma Lakshana, Bheda, Upadrava and Asadhya Lakshana of Atisara.
- ❖ Define Atisara. Explain the Nidana Panchaka of Atisara.

Short essays

- ❖ Explain the Samprapti and Samprapti Ghataka of Atisara
- ❖ Explain the Nidana and Samprapti of Atisara
- ❖ Explain Bhayaja and Shokaja Atisara
- ❖ Explain the types of Atisara
- ❖ Explain Asadhya Lakshana of Atisara
- ❖ Explain Upadrava of Atisara
- ❖ Explain Raktaja Atisara
- ❖ Explain Pravahika
- ❖ Explain the Nidana and Samprapti of Pravahika
- ❖ Explain Samprapti and Samprapti Ghataka of Pravahika

Short answers

- ❖ Define Atisara
- ❖ Write the Purvarupa of Atisara
- ❖ Enlist the types of Atisara
- ❖ What is Bhayaja Atisara?
- ❖ Write a short note on Shokaja Atisara

MCQs**Atisara**

1. **Dosha responsible for the manifestation of Pravahika is**
 - a. Vata and Kapha;
 - b. Vata and Pitta;
 - c. Pitta and Kapha;
 - d. Pitta and Vata

Answer: (a)

2. **Passage of mucus mixed stools with more amount of mucus is seen in**
 - a. Atisara;
 - b. Pravahika;
 - c. Grahani;
 - d. Gulma

Answer: (b)

3. **Types of Pravahika as per Madhavakara are**
 - a. 4;
 - b. 3;
 - c. 5;
 - d. 7

Answer: (a)

4. **Visrams is the synonym of**
 - a. Atisara;
 - b. Pravahika;
 - c. Gulma;
 - d. Grahani

Answer: (b)

5. **Visrams, Antargranthi, Nischaraka are the synonyms of**
 - a. Grahani;
 - b. Atisara;
 - c. Gulma;
 - d. Pravahika

Answer: (d)

Pravahika

1. Sankhya Samprapti of Atisara as per Charaka

- a. 4; b. 6;
c. 8; d. 3

Answer: (b)

2. Dosha dominance observed in the manifestation of Bhayaja Atisara is

- a. Vata; b. Pitta;
c. Kapha; d. Rakta

Answer: (a)

3. The description of Amaja Atisara is the contribution of

- a. Sushruta; b. Madhavakara;
c. Both a and b; d. Charaka

Answer: (c)

4. The type of Srotodushti occurring in the manifestation of Atisara is

- a. Atipravritti;
b. Sanga and Vimargagamana;
c. Siragranthi;
d. Sanga only

Answer: (a)

5. Atipravritti is the type of Srotodushti observed in

- a. Atisara; b. Prameha;
c. Asrigdara; d. All of the above

Answer: (d)

6. Which of the following effect will occur in water regarding Pakva Mala?

- a. Properly mixes; b. Sinks;
c. Floats; d. Foams

Answer: (c)

7. Which of the following is the cause of Atisara in general?

- a. Rasa Kshaya;
b. Rakta Vriddhi;
c. Svedavarodha;
d. Atyambupana

Answer: (d)

8. Varcho Bheda is the synonym of which of the following?

- a. Atisara; b. Kamala;
c. Visuchika; d. Pravahika

Answer: (a)

9. Typical rice water stools is seen in which of the following?

- a. Cholera; b. Food poisoning;
c. Cholitis; d. Gastritis

Answer: (a)

10. Dosha responsible for the manifestation of Pravahika is

- a. Vata and Kapha;
b. Vata and Pitta;

- c. Pitta and Kapha;
d. Pitta and Vata

Answer: (a)

11. Passage of mucus mixed stools with more amount of mucus is seen in

- a. Atisara; b. Pravahika;
c. Grahani; d. Gulma

Answer: (b)

12. Types of Pravahika as per Madhavakara are

- a. 4; b. 3;
c. 5; d. 7

Answer: (a)

13. Visrams is the synonym of

- a. Atisara; b. Pravahika;
c. Gulma; d. Grahani

Answer: (b)

14. Visrams, Antargranthi, Nischaraka are the synonyms of

- a. Grahani; b. Atisara;
c. Gulma; d. Pravahika

Answer: (d)

Chapter resources

- Charaka Samhita, Chikitsa Sthana – 19th chapter: Atisara Chikitsa Adhyaya
- Sushruta Samhita, Uttara Tantra – 40th chapter: Atisara Pratishedha Adhyaya
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- Textbook of Medical physiology by Arthur C Guyton and John E Hall, 11th edition, Saunders Elsevier publications, Philadelphia, 2006
- A manual on clinical surgery by S Das, 4th edition, published by Dr S Das, Calcutta, 1998
- Madhava Nidana – 3rd chapter: Atisara Nidana for Pravahika